

--SUMMARY--

Decision No. 2466/18

25-Sep-2018

L.Gehrke

- Consequences of injury

No Summary Available

7 Pages

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- WSIA

Other Case Reference

- [w4818n]

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 2466/18

BEFORE:

L. Gehrke: Vice-Chair

HEARING:

August 23, 2018 at Toronto
Oral

DATE OF DECISION:

September 25, 2018

NEUTRAL CITATION:

2018 ONWSIAT 3053

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated July 11, 2016

APPEARANCES:

For the worker:

R. Goodfellow, Paralegal

For the employer:

Not participating

Interpreter:

Not applicable

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
Toronto ON M5G 2P2

**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

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REASONS

(i) Introduction

- [1] The worker appeals a decision of the Appeals Resolution Officer (ARO), which denied ongoing entitlement; loss of earnings (LOE) benefits from September 11, 2015; and entitlement for a left hip labral tear as an area of injury on January 30, 2015.

(ii) Issues

- [2] The issues in appeal are as follows:
1. Ongoing entitlement beyond March 13, 2015 for the January 30, 2015 injury, including LOE benefits;
 2. Entitlement for anterosuperior labral tear of the left hip;
 3. Entitlement to LOE benefits from September 11, 2015.

(iii) Background

- [3] The following are the basic facts.

- [4] On January 30, 2015, the worker, who was then a 51-year-old beauty advisor, reported that she had slipped and fallen on the floor while walking to the lunchroom, injuring her lower back, left hip and left lower leg. She testified that she slipped on a freshly waxed floor and fell with her right leg forward and left leg backward and twisted. Her legs were completely flat on the floor, as in “splits.” She had never done this sort of movement before. She felt a pinching pain in the left leg and hip. The security guard heard her fall and came to assist her. She completed an accident report and sat in the lunchroom for about one-and-one-half hours, then continued to work with pain in her left leg, buttock and groin.

- [5] The worker testified that she first sought medical attention from her family doctor, Dr. O. Ayeni, MD and from her chiropractor. Her chiropractor had to stop treatment because she had too much pain. Her pain did not resolve after the injury on January 30, 2015. She testified that she had pain before the fall in her lower back related to standing. After the fall, the pain was where her leg attaches to her hip in the groin and at the back of the hip. It was not the same pain as before the injury. The pain was a constant pain which became sharp with certain movements. She had pain down her left leg and numbing in the foot. It became worse with walking and squatting.

- [6] The worker testified that after the fall, she could not go to the gym or swim as she did before. Her work duties were modified. She stopped receiving stock and she took more breaks. She did very little squatting unless she had to and her co-workers and manager helped her with her duties. At her request, her manager provided her with a compression mat.

- [7] After the fall, Dr. Ayeni prescribed Naproxyn and Tylenol 2. She tried massage, acupuncture, physiotherapy and Epson salt baths.

- [8] On September 11, 2015, her pain became so great when she squatted down to get a beauty product that she could not get back up and went to a hospital emergency department. This was not a movement that was different from the squatting she often did at work. She did

not work after this. She requested an MRI from her family doctor, but Dr. Ayeni refused. Soon after she stopped working, she was referred to a new family doctor, Dr. S. Hadcock, BSc, MSc, MD, CCFP, who immediately ordered an MRI and then referred her to a surgeon, Dr. F. Ayeni, MD, FRCSC. She was advised after the MRI that she had a cam type deformity of the left hip and a labral tear, for which she needed surgery. Dr. Ayeni advised her that she did not have arthritis in her left hip.

- [9] On August 17, 2015, the worker's physiotherapist, B. Balsor, BScPT, MCISc (Manip), FCAMPT reported to the worker's family doctor, Dr. O. Ayeni, that the worker had a "longstanding history of neck pain, previous MVA, had a slip and fall accident in January 2015 resulting in an aggravation of her neck and mid back and initiated a course of low back pain and stiffness..."
- [10] On August 24, 2015, Dr. Ayeni wrote to the employer requesting accommodation including a mat for the worker's "lumbar dysfunction and myofascial pains."
- [11] On September 11, 2015, the worker stopped working. Dr. Ayeni prescribed one month off work due to "continuing back pain."
- [12] On September 28, 2015, the worker's new family doctor, Dr. Hadcock, saw the worker for "ongoing back pain since injury at work at end of January." Dr. Hadcock observed spasm of the lumbar spine with decreased range of motion and pain; decreased internal and external rotation of the left hip and pain with palpation over the greater trochanter. Dr. Hadcock ordered an MRI of the left hip and lumbar spine.
- [13] On October 6, 2015, Dr. Hadcock assessed the worker and suggested that she "remain off work for the next 4-6 weeks to allow for ongoing therapy and while awaiting investigating/imaging."
- [14] On October 10, 2015, the worker's physiotherapist provided a Health Professional's Report (Form 8) describing the areas of injury on January 30, 2015 as the neck; upper and lower back; pelvis; and left hip, thigh, knee, lower leg and foot strain/sprain with decreased range of motion.
- [15] On November 4, 2015, an MRI of the left hip was performed with a clinical history of "Ongoing hip pain since January following a slip and fall." The MRI showed a "partial linear tear involving the anterosuperior labrum which may be on the basis of the cam type morphology of the femoral head and secondary impingement." Referral to an orthopaedic hip specialist was recommended. Dr. Hadcock referred the worker to Dr. F. Ayeni, on December 16, 2015 for assessment.
- [16] On November 19, 2015, Dr. Hadcock reported:
- This patient now has an MRI confirmed tear in her left hip and is awaiting a specialist appointment. I would recommend that she remain off work until after that assessment due to the physical nature of her work. This may take 2-6 months. ...
- [17] On December 11, 2015, Dr. Hadcock reported:
- I have reviewed some of [the worker's] old records before becoming my patient...It looks to me that most of the prior complaints were for her low back with some radiation to hip area on 1 visit. Particularly the visit of January 21, where she complained of L sided pain but patient states today that this was actually in the area of the low back/greater trochanter. The pain that she is complaining of at this time and since the

accident is deeper in the groin and the MRI would appear to confirm this and in my opinion, would be consistent with the mechanism of injury.

[18] On January 6, 2016, T. Anziano D.C. reported that the worker had attended his office for “intermittent low back pain since December 2009,” when she presented with an acute lumbosacral strain. A course of spinal decompression treatments was provided starting on January 7, 2015.

On Feb 2/15, the patient presented again for treatment. She reported feeling 50% better after the previous treatments, but she also reported a fall some days earlier that seemed to aggravate her pain. Pain locations appeared unchanged and there were no neurological signs.

During the treatment, the patient reported increasing back and hip pain to the point that treatment needed to be stopped. This had not happened with previous treatment. It represents a distinct change in her responsiveness to care that certainly could indicate a new injury, despite having pain in similar areas as previous episodes.

(iv) Law and policy

[19] Since the worker was injured in 2015, the *Workplace Safety and Insurance Act, 1997* (the WSIA) applies to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[20] Specifically, sections 13 and 43 of the WSIA govern the worker’s entitlement in this case.

[21] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[22] The standard of proof in workers’ compensation proceedings is the balance of probabilities. Pursuant to subsection 124(2) of the WSIA, the benefit of the doubt is resolved in favour of the claimant where it is impracticable to decide an issue because the evidence for and against the issue is approximately equal in weight.

[23] Pursuant to section 126 of the WSIA, the Board stated that the following Policy Packages, Revision #9, would apply to the subject matter of this appeal: #224 and #300.

[24] I have considered these policies as necessary in deciding the issues in this appeal.

(v) Analysis

[25] The appeal is allowed for the reasons set out below.

(a) Entitlement for labral tear of the left hip

[26] I find that the labral tear of the left hip resulted from the January 30, 2015 injury.

[27] The worker’s family doctor, Dr. Hadcock and Dr. M. Pysklywec, MD, MSc, CCFP(EM), DOHS, FCBOM, occupational medicine, who was consulted by the worker’s representative, are of the opinion that the MRI findings are consistent with the slip and fall she suffered on January 30, 2015. Dr. Hadcock’s review of the records indicated a new form of hip pain, located deeper in the groin, which is consistent with the worker’s testimony and with the MRI findings of a labral hip tear. Dr. Pysklywec wrote in part:

In summary, [the worker] was seen regarding the work-relatedness of her left hip condition. She has a tear of the labrum of the left hip. The onset of symptoms is reportedly from a work-related accident of June 30, 2015. There are some inconsistencies in the reporting.... However [the worker] tells me that her symptoms started from that accident. It is quite biomechanically plausible that such an incident could have caused a labral tear. There does however seem to be some underlying morphological issues of her hip that would predispose her to a labral tear. In particular, the MRI of 2015 describes a cam morphology of the hip that would predispose her to a labral tear. However x-rays do not describe this morphology.

This is a rather confusing clinical history and it is difficult to completely understand the onset of the condition. My feeling is that she had some predisposition to developing the labral tear based on the cam morphology of the hip. However ignoring the inconsistencies of the reporting and taking her at her word for the description of the accident, it would seem that she had an accident on January 30, 2015 which caused the acute development of a labral tear. It is certainly biomechanically plausible that this could occur.

- [28] The worker's representative submitted a medical article that was considered by the Vice-Chair in *Decision No. 54/18*, which allowed entitlement for a right hip labral tear on an aggravation basis: "A comprehensive review of hip labral tears" (M.M. Groh and J. Herrera), *Curr Rev Musculoskeletal Med* (2009) 2:105-117. That article states in part:

Because of the vast differential diagnosis and the need for specialized diagnostic tools, labral tears frequently go undiagnosed during an extended period of time. While magnetic resonance imaging (MRI) and computed tomography scans are unreliable for diagnosis, magnetic resonance imagery (MRA) is the diagnostic test of choice with arthroscopy being the gold standard. Typically, treatment begins conservatively with relative rest and non-steroid anti-inflammatory agents, with physical therapy (PT) being controversial. Often, surgical treatment is necessary, which entails, arthroscopic debridement of labral tears and surgical repair of associated structural problems.

- [29] The authors of the article, Groh and Herrera, describe the most common labral tears as in the anterior labrum (86%). There are at least five proposed etiologies: "trauma, FAI, capsular laxity/hip hypermobility, dysplasia, and degeneration..." With respect to cam morphology, they state:

Cam-type FAI is caused by an abnormal femoral head-neck junction, which leads to impingement between the abnormal femur and a normal acetabular rim through a peripherally increasing radius entering the acetabulum throughout ROM of the hip, especially flexion and internal rotation... cam FAI causes damage to the anterosuperior acetabular cartilage with separation between the labrum and cartilage. During flexion, the cartilage is sheared off the bone by the nonspherical femoral head, and the labrum remains initially untouched.

- [30] I note that the area of the labral tear in the worker's case is anterosuperior, consistent with the damage caused by a cam-type FAI as described by Groh and Herrera. The article also states that labral tears are "a frequent cause of anterior hip and groin pain" and commonly go undiagnosed for extended periods of time, with patients often seen by multiple health care providers before a definitive diagnosis is obtained. The average time for diagnosis is greater than two years. The onset of pain is described by the medical article as "insidious in 61% of patients. Many patients with labral tears describe a constant dull pain with intermittent episodes of sharp pain that worsens with activity."

- [31] I accept the worker's testimony that when she slipped and fell on January 30, 2015, she did the "splits," with her left leg fully outstretched behind her; and that she suffered ongoing pain

after the January 30, 2015 injury in her left hip that was of a different nature than before. I note that the worker reported left hip pain to her health care providers before the January 30, 2015 injury. However, she did not lose time from work due to left hip pain. In her reports to her health care providers and her testimony, she described the left hip pain as different from the pain she suffered afterwards, which was deeper in the groin.

[32] The opinion of the worker's treating and assessing physicians is that the diagnosed left hip labral tear is consistent with the mechanism of injury on January 30, 2015. I find that while the worker had left hip pain prior to the injury, this was of a different nature and was non-disabling. I further find based upon the medical opinion of the worker's treating and assessing physicians that the slip and fall injury on January 30, 2015 more likely than not resulted in the labral tear diagnosed in November 2015 by MRI. Applying the "thin-skull" principle, the fact that the cam morphology may have predisposed her to a labral tear does not bar entitlement, if the injury was a significant contributing factor to the tear. I find based on the mechanism of the injury, the history of symptoms and the medical opinions in the case materials that the January 30, 2015 injury was a significant contributing factor to the left hip labral tear.

[33] For all of the foregoing reasons, I find that the worker has entitlement for a left hip labral tear occurring on January 30, 2015 when she slipped and fell at work.

(b) Ongoing entitlement

[34] I find that based on her testimony, the worker did not recover from the injury of January 30, 2015. She suffered ongoing pain and limitation in her ability to perform daily living and work activities. She stopped working in September 2015 as a result of the injury. She remains off work as a result of the diagnosis of a left labral tear, while she awaits surgery. As a result, I find that the worker is entitled to ongoing benefits, including health care and LOE benefits, from the date of injury.

[35] Her entitlement to benefits includes LOE and health care benefits from March 13, 2015 and the date she stopped working in September 2015, including surgery to repair the left hip labral tear if required. The nature and duration of benefits are referred to the Board to determine subject to the usual rights of appeal.

DISPOSITION

[36] The appeal is allowed as follows:

1. The worker is entitled for a labral tear of the left hip occurring on January 30, 2015.
2. The worker is entitled to ongoing benefits including LOE and health care benefits from March 13, 2015 to September 11, 2015 and from September 11, 2015 onward, subject to any statutory reviews that apply.

[37] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: September 25, 2018

SIGNED: L. Gehrke